TRAUMA-05 | For therapists | Two separate age-based pathways

# Trauma-focused work guided by a manual

1 | Purpose, fit and boundaries

## Purpose of this unit

Record choices, questions and learning around work a skilled therapist has planned within the appropriate model. This unit does not teach processing itself or replace approved material from the full manual.

## Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

## Boundaries specific to this unit

There is no request to write or narrate the event, reenactment, exposure hierarchy or independent processing task. TF-CBT narrative and processing follow the model and professional plan. Written trauma accounts must not be mandatory by default in CPT.

## Reusing existing material

Unit 4 identifies meaning; unit 5 supports professional work chosen afterwards. TRAUMA-01 and regulation tools do not replace the focused component or its mapping in a full manual.

## A fictional example for discussion

Fictional example: Dana wants to understand the focused treatment component. The therapist explains options and boundaries, agrees on a choice and uses the sheet for questions, not the event story.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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# Working together with the worksheet

2 | Age-based pathways

## CH / TE | Support within a TF-CBT pathway

TF-CBT narrative and processing require the full manual and an adapted plan. CH/TE sheets explore choice, understanding and learning around the session only; they do not deliver this component.

## AD / OA | Support within a CPT pathway

Within CPT, AD/OA sheets record questions, understanding and follow-up chosen in conversation. They provide no self-processing instructions or required trauma narrative; manual-based work takes place separately.

## Parallel work with a suitable support person

Prepare a suitable supporter for supportive, attentive responses under the plan. Do not automatically disclose session details or assign home processing. Separate supporter materials remain a review need.

## Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

## Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

## Possible difficulties

Discomfort alone does not prove failure, and consent is ongoing. With confusion, dissociation or changed safety, the therapist assesses and adapts within professional care.

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# Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

## CH | Children, approximately ages 6 to 11

CH support sheets address session-related choice and understanding. They do not teach narrative or processing; the therapist needs the full manual, training and adaptation for child and supporter.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

## TE | Adolescents, approximately ages 12 to 17

TE respects independent voice, embarrassment and sharing boundaries. Do not require disclosure to a supporter or written details to prove cooperation; decisions occur within treatment.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

## AD | Adults

AD supports cognitive work chosen within CPT. This sheet does not require a written event account. Options and suitability are determined with the therapist according to the manual.

## OA | Older adults

OA preserves adult language and control over support choices. Assess fatigue, access and medical needs individually; age alone is no reason to withhold focused treatment.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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# Practice, review and next steps

4 | Shared choice and learning from the attempt

## Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

## At the next session

Review understanding and remaining needs without measuring success by memory detail or immediate distress reduction. Component, pace and continuation decisions rely on professional treatment and supervision.

## Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

## Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

## Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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